

DETAILED EXPERT REPORT

Child: Sumit (Male, 17 years 3 months)

Diagnosis on record: ADHD

Parent: Mr. Pradeep

Assessment date: April 2026

Reviewed by: Senior Paediatric Team, associated with Mr. Pradeep

EXECUTIVE SUMMARY

Sumit is a bright young man with exceptional cognitive strengths—outstanding general awareness, perfect vocabulary, excellent cardiorespiratory fitness, and remarkable language aptitude. However, we are deeply concerned about his current emotional state. He has expressed hopelessness and is engaging in self-harm, alongside profound difficulty identifying and recovering from emotions. These are urgent red flags that require immediate mental health intervention. Additionally, chronic sleep deprivation (6 hours nightly), social withdrawal, and academic underperformance in mathematics suggest he is struggling significantly beneath the surface. This is not simply ADHD—Sumit appears to be in genuine emotional distress that needs compassionate, expert support right away.

STRENGTHS WE NOTICED

✓ Exceptional intellectual capacity – Scored 88.5% in general awareness at maximum difficulty level, with perfect vocabulary and strong comprehension skills. This is well above age expectations.

✓ Outstanding physical health markers – Resting pulse of 72 bpm and 61-second breath-hold capacity indicate excellent cardiovascular fitness and respiratory control, far exceeding norms for his age.

✓ Natural language gift – Sumit shows rare talent in languages, memory, and storytelling—a powerful combination for future academic and career pathways.

✓ Emotionally safe home environment – Your parenting score reflects consistency, emotional safety, and modelling of calm behaviour, which are protective factors during this difficult period.

DOMAIN-BY-DOMAIN OBSERVATIONS

Behaviour (50.0%): Sumit has withdrawn from hobbies and interests outside academics, spends excessive time on screens that interferes with functioning, and shows erratic eating patterns. Most concerning: he has expressed hopelessness and self-harm thoughts within the past month. These are not typical ADHD features—they signal depression or significant emotional distress requiring urgent attention.

Emotions (62.5%): This is where we see the most worrying pattern. Sumit cannot name basic feelings, takes hours to recover from upset, does not seek comfort from you when distressed, shows limited empathy, and is engaging in self-injury. He has expressed feeling hopeless or worthless recently. This emotional shutdown combined with self-harm is a clinical emergency. At 17, he should be developing emotional vocabulary and regulation skills, not losing them.

Health (BMI 25.9, Sleep 6 hours): Sumit is mildly overweight for his height (170 cm, 75 kg), which is manageable with gentle lifestyle changes. The bigger concern is chronic sleep deprivation—6 hours is significantly below the 8-10 hours teenagers need. Poor sleep worsens ADHD symptoms, mood regulation, academic performance, and weight management. You've also flagged vision or hearing concerns that need formal assessment, as sensory issues can compound attention difficulties.

Mathematics (83.3%, Level 7): Sumit is performing 4-5 years below age level in maths, working at early-teen concepts rather than pre-calculus. This represents significant academic gaps, likely from years of ADHD-related inattention compounded by current emotional distress. The positive: 10/12 accuracy shows he can learn with proper support.

Language (100%, but slow reading at 75 wpm): Perfect vocabulary with full comprehension is excellent, but reading speed is half what we'd expect (150-200 wpm for his age). He's likely reading word-by-word rather than in phrases, which affects exam performance and study efficiency.

Speech (36.7%): The tongue-twister struggles reflect Hindi-English language transfer issues, not a speech disorder. His spontaneous storytelling was clear and confident. This is a language learning matter, not a clinical concern.

General Awareness (88.5%): Outstanding performance at maximum difficulty with quick response times. Sumit has a sharp, well-informed mind when he's engaged.

Mind Power (90%): Excellent cognitive processing abilities—this confirms his intellectual potential is intact.

Special Talents: Languages, memory, storytelling, leadership, and empathy are his natural gifts. The empathy score seems contradictory to the emotions module—this may reflect emotional shutdown rather than true lack of empathy.

Diet: The meal plan provided is sensible for gentle weight management, but eating patterns are currently erratic, which often accompanies emotional distress in teens.

PRIORITY CONCERNS

1. IMMEDIATE: Self-harm and suicidal ideation

Sumit has expressed hopelessness, feels worthless, and is engaging in self-injury. This is a psychiatric emergency. Depression and self-harm are common comorbidities with ADHD, especially in bright teens who feel they're underperforming. He needs professional mental health assessment within 48-72 hours.

2. URGENT: Emotional dysregulation and shutdown

His inability to name feelings, hours-long recovery from upset, and refusal to seek comfort suggest either severe depression, trauma response, or alexithymia (difficulty identifying emotions). This is not typical ADHD and requires specialized intervention.

3. Sleep deprivation

Six hours nightly is damaging his physical health, emotional regulation, ADHD symptom control, and academic performance. This must be addressed as part of his treatment plan.

4. Social isolation and loss of interests

Withdrawal from hobbies and relationships is a classic depression marker in teens. Combined with excessive screen time, this creates a negative cycle.

5. Academic gaps in mathematics

Four years behind in maths will affect board exam performance and college options. This needs structured remediation once his mental health stabilizes.

OUR RECOMMENDATIONS

1. URGENT MENTAL HEALTH REFERRAL (within 48 hours):

Contact a child/adolescent psychiatrist or clinical psychologist experienced with ADHD and depression. If Sumit expresses any immediate plan to harm himself, take him to the emergency department. Consider a safety plan at home—remove access to means of self-harm, increase supervision gently, and keep communication open without judgment.

2. Sleep hygiene overhaul (start tonight):

Establish a firm 10:00 PM lights-out rule aiming for 8 hours minimum. Remove all screens from bedroom 1 hour before bed. Consider melatonin 3mg at 9:30 PM after discussing with your paediatrician. Sleep is foundational to everything else.

3. Daily connection ritual (starting this week):

Spend 15-20 minutes daily in one-on-one time doing something Sumit chooses—a walk, tea, listening to music together. No advice-giving, just presence. This rebuilds trust and emotional safety when words are hard.

4. Vision and hearing assessment (within 2 weeks):

Book appointments with an ophthalmologist and audiologist. Undiagnosed sensory issues worsen ADHD and academic struggles.

5. Structured maths remediation (once mental health stabilizes):

After 4-6 weeks of therapy, engage a patient tutor who can systematically fill gaps from Class 8-9 level upward. Khan Academy or similar platforms for 20 minutes daily can help.

6. Physical activity routine (gradually introduce):

Aim for 30-45 minutes daily—walking, cycling, gym, or any activity he'll actually do. Exercise is evidence-based treatment for both ADHD and depression. Start small and build consistency.

7. Weekly strength-naming:

Explicitly tell Sumit one specific thing he did well each week: "I noticed how you explained that concept clearly" or "Your general knowledge is genuinely impressive." Depressed teens cannot see their own strengths—you must reflect them back.

WHEN TO CONSULT A SPECIALIST

IMMEDIATELY (this week):

- Psychiatrist or Clinical Psychologist for self-harm, hopelessness, and emotional dysregulation. Ask specifically about depression screening and whether ADHD medication needs review.

WITHIN 2 WEEKS:

- Ophthalmologist for vision concerns

- Audiologist for hearing assessment
- Paediatrician for overall health review, sleep management, and weight guidance

IF symptoms worsen:

- Emergency Department if Sumit expresses any plan to harm himself or shows sudden behaviour changes
- Neurologist if headaches, seizures, or neurological symptoms emerge (discuss with paediatrician first)

AFTER mental health stabilizes (6-8 weeks):

- Educational psychologist or learning specialist for maths remediation strategies
- Occupational therapist if ADHD-related executive function difficulties persist despite treatment

PERSONAL NOTE FROM OUR TEAM

Mr. Pradeep, Sumit is clearly a young man of exceptional ability who is currently drowning emotionally. The good news: he has you—a medically trained, emotionally attuned parent who has already created a safe home environment. With prompt mental health intervention, improved sleep, and consistent connection, we have every reason to believe Sumit can emerge from this dark period. His intellectual gifts are intact; we just need to help him find his footing emotionally. Please act on the mental health referral immediately—this is the most important step. You're doing the right thing by seeking comprehensive assessment. We're here to support you both.